SETTING UP MEDBURY AESTHETICS

Lagos first. Five businesses, one division. What the work is, how it is measured, and what it costs.

Itunu,

You have offered NGN 15M and I came back at 25. Rather than split it, let me show you what is actually in the work, because I do not think the word setup is helping either of us. It sounds like registration and paperwork. It is not.

The number you were working from was the Abuja document, where NGN 45M covered the project management fee on the facility build as well. In Lagos your own contractors may well manage that build, and if they do I should not be charging you to manage it. Strip that fee out and what is left is the harder half: the operating build. One rate card, one procurement framework, one protocol set and one patient record across five businesses, with every service partner found, negotiated and under a signed service level agreement before a single door opens.

That is five businesses, not one. And almost none of it is a Lagos asset: the rate card, the protocols, the record, the partner terms and the procedures are written once and transferred to Abuja when it is ready. Build it cheaply here and Abuja has to be build the same thing a second time.

I have also put the government and regulatory fees inside the number rather than on top of it. Across five entities they are about NGN 3M, so what CFA actually earns is under 22. You should not be receiving invoices for filing fees from me.

So I have written the scope out, tied the fee to key performance indicators you can hold me to, and set it out as one arrangement rather than two: three months to build, two months to move it onto the management rhythm, and then CFA runs the division from month 6 on the management terms we have now agreed. If I miss the measures, you do not pay the tranche. That is the part I would hold me to if I were you.

Debo Odulana
Founding Partner, Consult for Africa

01 / WHAT IS BEING SET UP

Five businesses, deliberately run as one division.

Medbury Aesthetics is not a clinic. It is five separate operating companies that share a market, a supplier base and a patient. Lyfe Plastics and Dermatology is one of the five and already has its own memorandum and its own shareholders. The other four are yours.

Four of the five are built from nothing. Only Medlyfe Regenerative and Longevity is trading today and it is nascent, so there is very little book to inherit and almost no patients to start from. That is exactly why the cross-sell between them has to be designed in from the first day rather than hoped for later. A patient acquired once should be reachable by four more businesses without being acquired again, and that only happens if the record, the rate card and the referral path were built to allow it. The regenerative book, small as it is, is the one warm list the division starts with and it should be the first thing the cross-sell is pointed at.

Entity	What it is and why it is in the division
Medlyfe Regenerative and Longevity	Regenerative medicine, longevity programmes and infusion. Already trading but nascent, and the only one of the five with patients today. Programme based and pre-sellable, so revenue can be taken before the doors open. Highest revenue per square metre on the campus
Bariatric and Metabolic	Medical weight management. Long programmes, high retention, feeds pharmacy and laboratory directly
Cosmetic Dentistry	Veneers, whitening, alignment and implants. High ticket, cash paying, and the most natural cross-sell in the division: the patient having her face treated wants her smile to match
Hair Restoration	Single patient days, instruments brought by the operating partner, very little fixed cost
Lyfe Plastics and Dermatology	The surgical end, with medical and surgical dermatology alongside it. Its own tri-party shareholding, its own clinical director, its own memorandum, already drafted

The whole commercial argument for the division is that these five share. One procurement framework buys devices and non-clinical supply once, not five times, and routes the clinical supply to the group's own companies. One rate card stops five businesses pricing against each other in the same building. One patient record means the woman who arrives for one of the five is known to the other four, without being acquired a second time. Set them up separately and you get five small clinics with five cost bases. Set them up as a division and the second, third and fourth cost almost nothing to add.

02 / WHAT THE WORK ACTUALLY IS

Not registration. The operating build.

Company formation is a week of somebody's time and a few hundred thousand naira of government fees. It is not what you are buying and I would not charge for it as though it were. This is the work.

Built once, and every entity inherits it

Workstream	What is delivered
The operating model	How five businesses share one reception, one record, one booking spine and one back office without stepping on each other. Who owns which patient, how a referral moves between them, how the revenue is attributed
The rate card	One pricing architecture across all five, researched and sensitivity tested rather than copied. Set out in full at section 04 because it is the largest single item of value in this mandate
Supply and demand routing	The division's consumables, dispensing and laboratory demand specified once and routed to Medbury's own pharmacy and diagnostics businesses, with service levels between group companies. Any margin there stays in the group, which is why it is not claimed as value at section 07
Clinical governance	Protocol set per entity, credentialing standard, complications and escalation, incident reporting, consent architecture. Written so a regulator, an insurer and a plaintiff's lawyer all find the same answer
The technology platform	One record, one diary, one till and one dashboard across five companies, configured and live before opening. Set out at section 03 because it is what makes the division a division rather than five clinics at one address
The funnel	Channels, enquiry handling, conversion, and the content engine. Built once and pointed at five menus
Front office and revenue cycle	The path from enquiry to booking to deposit to collection, written down and enforced. Deposit rules, the no-show and cancellation policy, instalment and refund terms, and who chases what. In a cash paying premium business this is the difference between revenue and a receivables ledger
The governance rhythm	The board pack, the management dashboard and the operating cadence. What the Board sees each month, what the division reviews each week, and which numbers trigger a decision rather than a discussion

Built per entity

Per entity	What is delivered
Scope and menu	What each entity sells and, as importantly, what it does not
Staffing model	Establishment, grade mix, rota, and what each role costs fully loaded
Equipment specification	Specified against this patient population rather than a vendor's catalogue. Several of the best known devices are the wrong instruments for the skin this division will treat, and buying by brand recognition is the most expensive mistake available here
The clinical brief to the build	What the space has to do before anyone draws it. Room adjacencies, the patient path and where it must not cross a staff or soiled path, privacy and discretion for a clientele that expects it, recovery, and the power, water and gas the clinical rooms depend on. Your contractors build it. This is what they build to, and it is cheaper to say now than to move a wall later
Launch sequence	The order the five open in, so that the earliest cash funds the later builds
Commissioning and opening readiness	Before the first real patient: equipment accepted and calibrated, the platform run end to end on mock bookings, dry runs with the actual staff through the actual rooms, snagging closed, and a signed go-live checklist per entity. An opening date is not a readiness test

03 / THE TECHNOLOGY PLATFORM

Five businesses cannot run as one division on paper.

Everything claimed for the division in section 01 depends on software that most clinics of this size do not have. One rate card is a document until something enforces it at the point of booking. One patient across five businesses is an aspiration until one record holds her. Programme revenue cannot be taken before opening unless something can sell a course of treatment, collect for it and track what has been delivered against it. This is a platform build, not a software purchase, and it is configured and implemented as part of the mandate rather than bought in afterwards.

What it has to do	Why the division depends on it
One clinical record across five companies	Shared demographics and history, entity-level clinical notes, so a patient treated by one is known to the other four without her file being duplicated or her consent assumed
Scheduling across shared rooms and diaries	Five businesses competing for the same rooms, chairs and clinicians. Room and resource booking, not just appointment slots, or the division loses its capacity advantage on day one
Programme and course management	Sell a course of treatment, collect for it in full or by instalment, and track sessions delivered against sessions paid. This is the machinery behind the pre-opening revenue at section 07, and without it that revenue cannot be recognised or refunded safely
Billing, deposits and collections	Deposits at booking, instalments, refunds and credit notes, reconciled to the payment gateway. Cash paying and premium means collection discipline is the difference between revenue and receivables
Clinical imaging and consent capture	Standardised before and after photography held against the record, with the consent to treat and the separate consent to publish captured, versioned and revocable. In aesthetics this is both the marketing asset and the legal exposure
Inventory, cold chain and expiry	Batch and expiry tracking for injectables and consumables, cold chain exceptions logged, and usage tied to the procedure so cost of care is real rather than estimated
The enquiry funnel and customer relationship	Every enquiry captured, sourced, followed up and converted, so cost per booked patient is measurable rather than assumed. The cross-sell between the five is executed here
Reporting and the board pack	Management accounts, clinical quality and complications, utilisation, conversion and pipeline, produced from the system rather than assembled by hand each month

> **This is where the division is won or lost.** Five clinics each with its own diary, its own spreadsheet and its own cash box is what Medbury would get by default, and it makes every claim in section 01 untrue. The platform is what turns five companies into one operating business, and it transfers to Abuja already configured.

CFA's own platforms are licensed to the division for the term and the configuration, the data and the integrations belong to the company. What is built here is not a demo. It is the system the five businesses run on from the first patient, and the staff are trained on it before opening rather than during.

04 / THE PRICING WORK

Researched, costed and sensitivity tested. Not copied.

This is the largest single item of value in the mandate and the one most often done badly, because a rate card looks like something you can assemble in an afternoon from a competitor's website. Done properly it is six weeks of work and it sets the margin of five businesses for years. Done badly it is unrecoverable: you cannot raise prices on a patient base you acquired at the wrong number.

What is actually done

Step	What it involves
Primary price research	Every comparable provider in Lagos priced at the point of enquiry rather than from a website, because published prices and quoted prices are not the same thing. Covering the premium clinics, the mid market and the diaspora alternative, which for several of these lines is a flight
Willingness to pay	Structured testing with the segment this division is actually for, on price points and on what moves a patient from enquiry to booking. Programme pricing tested separately from single treatment pricing, because they behave differently
Cost to serve, bottom up	Every procedure costed from its own consumables, clinician time, room time and share of fixed cost. Not a percentage applied across a menu. This is what tells you which treatments are worth having on the list at all
Margin architecture	Target gross margin set per entity and per line, with the loss leaders identified deliberately rather than discovered later. Some treatments exist to fill a diary and some exist to pay for it, and the two should not be confused
Programme and bundle design	The programme structures that let regenerative, longevity and weight management be sold and paid for before delivery. This is what creates the pre-opening revenue at section 07, so it is priced with the cash profile in mind and not only the margin
The published card	The final rate card, the internal margin model behind it, the discount authority matrix, and the review cadence that keeps it current

The sensitivity testing, which is the part that is usually skipped

A rate card that only works at the planned volume, the planned mix and today's exchange rate is not a rate card, it is a hope. Each of these is modelled and the card is set so it survives them.

Tested against	What is modelled
Volume	At sixty, eighty and one hundred per cent of planned fill. The card has to clear contribution at the low case, because the low case is what year one looks like
Exchange rate	Consumables and devices are imported and priced in dollars. Modelled across a realistic naira band, with the trigger points identified at which a price has to move rather than a margin quietly disappear
Mix	If the high margin lines underperform and the volume lines carry the division, does it still work. This is the most common way an aesthetics plan fails
Price elasticity	What a ten per cent move in either direction does to volume and to contribution, so the discount authority given to the front desk is bounded by evidence
Ramp	Break-even by entity and by month, so it is known before opening which of the five carries the division early and which take longer to stand up

What you get at the end of it. A rate card, the model underneath it that shows how every number was reached, and the break-even position of each of the five entities. All three are yours, they transfer to Abuja, and they are the reason the pricing line at section 07 is worth more than the fee on its own.

05 / THE SERVICE PARTNERS

Found, negotiated, and under a signed SLA.

A division like this does not own everything it needs. Each of these is a search, a negotiation and a contract, and each carries a service level agreement with a measure in it, because a partner without a measure is a promise.

Laboratory, pharmacy and imaging are not on this list. Medbury owns those businesses and the division should be their customer, not the market's. The work there is different and it is still work: specifying what the division needs, routing its demand internally, and writing service levels between group companies so that the newest and smallest customer is not the one served last. That is an internal negotiation rather than an external one, and the margin stays in the group.

Partner	What is contracted	The measure in the SLA
Partner theatre and anaesthesia	Theatre access and scheduling, anaesthesia, recovery, escalation and admission, per-case fees and the flow of funds	Case slots available within X days; escalation answered in minutes not hours
Specialist operating partners	The hair restoration and dental operating partners, without whom two of the five entities do not exist. Sourced, credentialed and contracted	Named, credentialed and contracted before that entity is committed to
Equipment service and maintenance	Devices, autoclave, power	Response and restore times, planned maintenance calendar
Medical indemnity and insurance	Cover for the division, each entity and each clinician, plus the premises	Cover in force before any patient is seen, evidenced annually
Clinical waste	Collection, documentation, compliance	Collection frequency and consignment notes
Marketing and content	Production, media buying, the enquiry desk	Cost per qualified enquiry and conversion to booked

Identifying a partner is the easy part. The value is in what the agreement says when the partner underperforms, and in having a named second source for anything that can stop a list. Every one of these arrangements is contracted by the company rather than by any individual, so the relationship belongs to the business and not to a person.

06 / BUILT ONCE, USED TWICE

Lagos builds the operating system Abuja inherits.

This is the part of the price that is easiest to miss, and it is the reason I would rather you paid properly for it now than cheaply twice. Almost everything in sections 02 to 04 is not a Lagos asset. It is a division asset. It is written once, and when Abuja is ready it is transferred rather than rebuilt.

What is built in Lagos	To Abuja	Why
The rate card and pricing architecture	Transfers whole	Abuja adjusts for local willingness to pay. The architecture, the programme logic and the pre-sell model do not change
The procurement framework	Transfers whole	Device and equipment terms negotiated on two-campus volume, and the internal routing to the group's pharmacy and diagnostics already specified. Abuja inherits both

What is built in Lagos	To Abuja	Why
Clinical protocols and governance	Transfers whole	Same protocols, same credentialing standard, same complications and escalation framework. A clinician trained in one is competent in the other
The patient record and its configuration	Transfers whole	One record across two campuses, so a patient known in Lagos is known in Abuja
SLA templates and partner terms	Transfers as templates	The agreements are re-papered with Abuja partners, but the terms, the measures and the negotiating position are already won
Standard operating procedures	Transfers whole	Reception, booking, consent, stock, cold chain, incident reporting, month end
The funnel and content engine	Transfers with local media	Channels, enquiry handling and conversion are the same machine pointed at a different city
Facility, team and licences	Does not transfer	Local to each campus, and correctly excluded from this fee

So the fee is not really a Lagos fee. It buys the operating system for the division, and Abuja inherits it for the cost of re-papering, which is why Abuja sits inside this fee rather than behind a second one. Set it up cheaply here and there is nothing to transfer, so Abuja has to be built a second time. Judged across both campuses this is the least expensive way to arrive at two.

07 / WHAT THIS IS WORTH

Four numbers, each of which outlives the fee.

A setup fee is only defensible if it returns more than it costs, and in a way the Board can check afterwards. These are the four places the money comes back. They are also the targets the fee is measured against at section 08, so this is not a brochure claim: it is what I am asking to be held to.

Where the value is	NGN M	How it arises
The pricing architecture	20	A rate card built on what this market pays, with the programme and bundle logic designed in, against one built by copying a list. 5 points of margin on modelled division revenue of NGN 400M. It is set once and it compounds every year afterwards
Revenue before opening	25	Regenerative, longevity and weight management are sold as programmes, so they can be contracted and paid for before the doors open. No other clinical model in the group lets you bank revenue ahead of trading
The Abuja build not repeated	18	The rate card, the procurement framework, the protocols, the record and the procedures transfer. Abuja is re-papered rather than rebuilt, which is what lets it sit inside this fee. Section 06
Cross-sell between the five	recurring	A patient acquired once is reachable by four more businesses at no further acquisition cost. Over time the cheapest revenue in the plan, and it only exists if the division is designed for it at the start
Against a fee of	**25**	**The first three alone exceed it inside year one, and two of them repeat**

Supply, pharmacy and diagnostics are deliberately absent from this table. Medbury already owns those businesses, and any saving or margin the division produces there belongs to the group rather than to me. I will put the division's consumables, dispensing and laboratory demand through your own companies and write the service levels that make them work, but I am not going to present internal capture as value I brokered. It is a real upside and it is entirely yours.

The honest version of this table. Every figure in it is a target I am proposing, not a promise I have banked. That is precisely why they appear again in the next section as the measures that release the tranches. If the pricing work does not defend itself, the tranche does not get paid. I would rather be paid for what these produce than for the meetings that produced them.

08 / THE SPRINT

Three months, with the measures that release the money.

The old version of this page measured activity: suppliers approached, documents approved, models signed off. None of that is worth paying for. These measure outcomes, in naira or in dates, and each one is verifiable by someone who was not in the room.

When	NGN M	What has to be true
On signing	15	**Mobilisation.** The team stands up and is paid from the first week, the government and regulatory fees CFA absorbs start falling due, and every workstream opens at once because the sequence does not allow them to run one after another. This is the only tranche not tied to an outcome, and it is the cost of starting
1		**A rate card that survives its sensitivities.** Delivered with the primary price research behind it, the bottom-up cost to serve per procedure, and the model showing each entity still clears contribution at sixty per cent of planned fill and across a realistic naira band. A card without that model attached does not count as delivered
1		**Internal supply routed and specified.** The division's consumables, dispensing and laboratory demand specified and routed through Medbury's own pharmacy and diagnostics businesses, with service levels written so the division is not the customer they serve last
2		**Cash before opening.** At least NGN 25M of programme revenue contracted and collected from regenerative, longevity and weight management before the first door opens
2		**Supply that cannot stop a list.** Primary supplier and a named, quoted second source signed in every critical category, with lead times in the agreement
3	5	**Trading.** First entity lawfully open and first revenue banked, with every service partner under a signed SLA carrying a numeric measure and a remedy. This tranche is released only if *every* measure above it has also been met, so one missed milestone holds the whole of it
3		**Cross-sell working.** An agreed proportion of patients treated by the first trading entity booked into a second within ninety days, measured from the record
6	5	**At the switch to management.** The handover is complete, the division is running on the normal management rhythm rather than the sprint, and the measures above are still being met. It is easy to hit a number once. This payment is for it holding, and it falls due on the day the management agreement starts
Total	**25**	

Three payments, and the shape is deliberate. NGN 15M to start, because the team is assembled and paid before a single measure can be met and the sequence needs every workstream open at once rather than in turn. NGN 5M when the division is trading, and only if every measure has been met. NGN 5M at the switch to management in month 6, and only if it is still trading to the same measures once the sprint is over. Certainty on the cost of starting, and the whole of the rest at risk.

09 / THE HANDOVER, AND THEN MANAGEMENT

Two months of transition, and then CFA runs the division.

We have agreed that CFA manages Medbury Aesthetics once it is built. So this is not a setup that ends and hands over to a stranger. The two months are a transition between two phases of the same relationship, and that changes how the build is done: I am building something I then have to run, which is the strongest guarantee available that it is built properly.

- **Month 4.** The division moves from build to operation. Every function has a written operating pack: the procurement framework with its terms, the SLA file, the protocol set, the platform administration, the rate card and its logic, the funnel and its numbers.
- **Month 5.** The division runs on the normal management rhythm, with the board pack produced from the platform and the weekly operating review in place. The last of the setup fee is released only if the measures are still being met.
- **Month 6.** The management agreement starts on its own contract, with the KPIs that sit behind it agreed before it begins rather than after.

The management agreement		What it means
The two chief operating officers	Medbury employs and pays	CFA recruits both at its own cost, Medbury approves each appointment, and both are seconded to CFA and report to CFA for delivery
Support fee	NGN 3.0M a month	The CFA back office, and the direction of the division toward the goals the Board sets. Payable in advance on the fifth business day of each month from month 6
Performance fee	3% of collected net revenue	Cash actually collected, net of VAT and refunds, and net only of items passed through at cost. One element only, with no share of EBITDA. Payable only where the Board's revenue and EBITDA goals are both met, so it is not paid on revenue bought at the expense of profit
Term	24 months	Reviewed at 12 months, with Medbury's right to terminate for sustained failure against the agreed measures

In practice	What it means for Medbury
What it covers	A chief operating officer inside the division, and the whole back office behind that person sitting at CFA rather than on your payroll: finance and revenue cycle, marketing and communications, clinical governance and quality, platform administration and the board pack, recruitment, procurement and the rate card kept live. The division pays only for the front line, meaning everyone who touches a patient, plus stock, licence and marketing spend
Why most of it is variable	Because it puts CFA on the same side of the table as the Board. Two of the three parts pay nothing unless cash is collected or profit beats budget, and the fee is on collections rather than billings, so a division that bills well and collects badly pays me nothing on the difference
What it is worth to Medbury	An operating team for a five business division without carrying one post of it on your payroll or one desk of it in your building. On modelled revenue of NGN 400M that is about NGN about NGN 36M a year of support fee for the whole CFA back office and the direction of the division, against five or six salaries and the desks they sit at if you hire the same capability and carry it every month, trading or not

The setup fee and the management agreement are one arrangement, not two. CFA prices the build at NGN 25M, which is well below what the work costs on its own schedule, precisely because CFA runs what it builds afterwards. If the management agreement is not part of this, the setup is a different piece of work at a different price, and I would say so rather than pretend otherwise.

10 / WHAT THE DIVISION COSTS TO BUILD

The decision underneath the fee, and the one nobody has put a number to yet.

This has been discussed as a fee. The fee is the small number. The decision actually in front of the Board is the capital behind five clinical businesses, and it has not been put on a page yet. It belongs here, because a setup mandate that does not tell you what the setup costs is not a mandate, it is an invoice.

	NGN M	Basis	What drives it
Lyfe Plastics and Dermatology	126	Worked	Modelled in the memorandum already: consulting and minor procedure rooms, surgical kit, stage one costs and working capital through the ramp
Aesthetic devices, first tranche	105	Worked	Three devices specified against Fitzpatrick IV to VI skin rather than a vendor's catalogue. Shared across dermatology and the aesthetic menu
Cosmetic Dentistry	95 to 145	Indicative	The most capital intensive of the five. Chairs, imaging, intraoral scanning, milling and the compressed air and suction behind them
Medlyfe Regenerative and Longevity	55 to 75	Indicative	Infusion bays, cold chain, compounding, and an opening stock of injectables. Trading already but nascent, so part of this is a step up rather than a standing start
Bariatric and Metabolic	55 to 85	Indicative	Light on equipment. The opening drug stock is the largest single item and it is cold chain
Hair Restoration	18 to 28	Indicative	The lightest of the five, because the operating partner brings the instruments
Division shared: front of house, plant, platform hardware	25 to 40	Indicative	Reception, records, waiting, power backup and the terminals the platform runs on
Equipment, pre-opening and working capital	**479 to 604**		**Fit-out sits outside this and is priced by your own contractors**

Two of the seven lines are worked and five are indicative. Lyfe Plastics and Dermatology and the device tranche are modelled already, to the line. The other four entities have never been costed by anybody, and costing them is the first thing the sprint produces, because specifying equipment against this patient population is one of the deliverables. Treat the range as the order of magnitude, not the budget.

	Why it matters
What the fee is, as a proportion	NGN 25M against NGN 479 to 604M of capital. Between four and five per cent of the money at risk, spent on deciding where the rest of it goes
Why the order matters	The capital budget is an output of the sprint, not an input to it. You are not being asked to commit the capital to approve this mandate. You are being asked to buy the work that tells you what the capital is, before you commit it
Where it goes wrong without this	Five businesses buying equipment separately, on vendor advice, against catalogues written for other skin. One wrong device is tens of millions in a room, and the cosmetic dentistry line alone can be specified fifty million apart by two honest people

The first tranche of the sprint delivers the capital budget. Entity by entity, specified, quoted and sequenced against the launch order, so the earliest cash funds the later builds. That is the deliverable that turns the range above into a number the Board can approve.

11 / WHAT IT COSTS

What the fee covers, and how it sits against Abuja.

Against that capital, this is the fee and where it comes from. The Abuja figure was NGN 45M and it included the project management fee on the facility build. In Lagos that build may well be managed by your own contractors, in which case there is no fee for me to charge, and there is none in this price.

	NGN M	Note
Abuja, as quoted	45	Included CFA project management on the facility build
Less that project management fee	(20)	Your contractors manage the Lagos build, so there is nothing for CFA to charge
Comparable scope	**25**	**The operating build, across five entities rather than one**

CFA absorbs the government and regulatory fees. Incorporation, tax registration, facility registration, clinical and practitioner registrations, pharmacy and dispensing permissions, controlled drug handling, data protection and the trade mark filings across all five entities come to roughly NGN 3.1M. They are inside the NGN 25M rather than billed on top, so the net fee to CFA is under NGN 22M. I would rather absorb them than have you receive a schedule of small government charges and have to wonder what else is coming.

The three ways to test the number

A day rate is the wrong test and I am not going to offer one. What matters is what the fee produces, what it prevents, and what the alternative costs.

Test	What it shows
Against what it produces	Section 07, not repeated here: NGN 63M of identified year-one value against a fee of NGN 25M, two lines of which recur
Against what it prevents	A rate card set five points light on division revenue of NGN 400M costs NGN 20M a year and cannot be corrected, because you do not raise prices on a base you acquired at the wrong number. One quarter of delay across five businesses costs a full quarter of their revenue, permanently. One wrong device is tens of millions of naira of capital in a room, unused
Against building it yourself	The capability is a division lead, a pricing analyst, a platform implementer, a regulatory hand and a recruiter. Resourced honestly that is close to three full time people for five months. Hire them and you are above this fee before anyone has negotiated anything, you carry them after the build is finished, and you have nothing written down to transfer to Abuja

The test I would apply if I were you. Hold the whole fee against the rate card alone. If the pricing work at section 04 does not defend its own margin assumption under the sensitivities, on evidence you can read, then nothing else in this mandate was worth buying either. I am content for that to be the measure, and it is why the pricing tranche is the first one and the largest.

12 / WHAT MEDBURY DOES

So that nothing waits on an unnamed decision.

Medbury provides	Why it matters
A single decision maker	One person who can approve a rate card, a supplier and an SLA without a second meeting. Most setups lose more time to waiting than to work
The facility	Whoever builds it, on a programme CFA can plan the launch sequence against
The funding	Against the approved budget, drawn as the sequence requires
The operating lead	CFA recruits and the Group approves, in post by month 4, so the person who will run the division is chosen for it
Group access	Referrals, laboratory, pharmacy and the relationships that open a partner theatre faster than a cold approach

13 / THE BOUNDARY

What this fee does not cover.

- Project management of the facility build and fit-out, which is why this is 25 and not 45. If Medbury would rather CFA managed that build after all, it is quoted separately and it is not in this number.
- Equipment and stock, which is capital and sits in the company's budget.
- Hardware, connectivity and third party subscriptions behind the platform. The build, the configuration, the migration and the training are in the fee. The terminals, the network and the payment gateway charges are the company's.
- Salaries of the front line team recruited during the sprint, meaning everyone who touches a patient, who are the company's employees from day one. Under the management agreement that follows, the back office behind them sits at CFA and not on your payroll.
- The shareholder negotiation for Lyfe Plastics and Dermatology, which is separately mandated and already in hand.
- The management of the division from month 6, which is agreed and sits on its own contract rather than inside this fee.

14 / DECISIONS

Four, and then we start.

Decision	What is being decided	Whose
The fee	NGN 25M against the tranches and measures at section 08	Yours
The start date	The sprint is three months from mobilisation and the sequence is tight. A start in the next fortnight puts first revenue inside the year	Yours
The management agreement	Terms agreed and issued separately. What remains is the contract, the KPI schedule and the budget process, all drafted during the sprint rather than at the end of it	Joint
The operating lead	Recruited by CFA during the sprint and in post by month 4, so that the person who runs the division was chosen for it rather than inherited	Mine

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Prepared for Dr Itunu Akinware, Group Chief Executive, Medbury Healthcare Group. Private and confidential. Capital figures at section 10 are worked for two lines and indicative for five, and are firmed in tranche one of the sprint.